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JUSTICE K.S. HEGDE CHARITABLE HOSPITAL

University Road, Deralakatte - 575018

Tel: 0824 2204471-76 (6 LINES)

Laboratory Test Result

Register No.: 19026933

Report No.: L1727963

Patient Name: SHANTHARAMA VENKATRAMANA HEGADE

Age: 65 Years Sex: Male

Location: OPD Day

Dept/Unit: E.N.T/Unit 1

Bill/Advice No.: 50075236

Lab ID/Slide No. : S/5559/19

Collection Date & Time : 22/6/19

Observed Date & Time: 25/6/19 12:54 PM

PATHOLOGY

Histopathology

CLINICAL DETAILS :

C/o pain in oral cavity and c/o throat pain.

Proliferative growth in right RMT.

? Malignancy right buccal mucosa.

GROSS :

Received pale white to pale brown tissue bits largest measuring 0.3x0.2cm. All embedded in 5559.

MICROSCOPY :

Right buccal mucosa biopsy - Sections studied show an extensively ulcerated squamous epithelium with cells displaying high grade dysplasia with hyperchromatic nucleus and scant cytoplasm. The subepithelium is scant and show focal nests of pleomorphic cells with high N:C ratio, hyperchromatic nucleus and scant cytoplasm. Also noted subepithelial brisk lymphocytic inflammatory infiltrate.

Impression :

Histological features are suggestive of Carcinoma in-situ with early invasion - Right buccal mucosa.

Entered By: KRIPA


Dr. Jayaprakash Shetty K. M.D(Path)
Professor & HOD, Pathology


Dr. Krishna Prasad H.V, DCP, M.D (Path)
Assistant Professor

Printed On: 25-06-2019, 01:03 PM

Report Generated On: 25/6/19, 12:54 PM

REPORT WITHOUT CONSULTANT SIGNATURE IS AN INTERIM REPORT

(4)

JUSTICE K.S. HEGDE CHARITABLE HOSPITAL
University Road, Deralakatte - 575018
Tel: 0824 2204471-76 (6 LINES)
Laboratory Test Result

Register No.: 19026933

Patient Name: SHANTHARAMA VENKATRAMANA HEGADE

Dept/Unit: E.N.T/Unit 1

Report No.: L1738405

Age: 65 Years Sex: Male

Location: OPD Day

Bill/Advice No.: 50089555

Lab ID/Slide No.: 514/19

Collection Date & Time: 05/7/19 10:30 AM

Observed Date & Time: 05/7/19 02:31 PM

PATHOLOGY

Fine Needle Aspiration Cytology (FNAC)

Right Cervical lymphnode

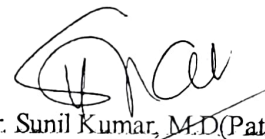
Smears studied are highly cellular and show pleomorphic squamous cells arranged in singles and tiny clusters, sparse lymphocytes against haemorrhagic background.

Impression :

FEATURES ARE SUGGESTIVE OF METASTATIC SQUAMOUS CELL CARCINOMA - RIGHT CERVICAL LYMPHNODE.

(Cytology slides will be preserved for five years)

Entered By: GLADYS CRASTA


Dr. Sunil Kumar, M.D(Path)
Additional Professor

Printed On: 05-07-2019, 02:33 PM

Report Generated On: 05/7/19, 02:31 PM

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JUSTICE K.S. HEGDE CHARITABLE HOSPITAL

University Road, Deralakatte - 575018

Tel: 0824 2204471-76 (6 LINES)

Laboratory Test Result

Register No.: 19026933 IP No.: IP009190

Patient Name: SHANTARAM VENKATRAMAN HEGDE

Report No.: L1776272

Age: 65 Years Sex: Male

Location: General Ward/C9

Dept/Unit: E.N.T/Unit 1

Bill/Advice No.: A0086200

Lab ID/Slide No.: S/6795/19

Collection Date & Time: 29/7/19

Observed Date & Time: 05/8/19 02:36 PM

PATHOLOGY

Histopathology

CLINICAL DETAILS: C/O Burning sensation in mouth since - 2 months.
SCC of right buccal mucosa.

GROSS: Received 2 containers.

Container labelled (2) Right MRND + sternocleidomastoid - Received 4 fibrofatty tissue mass larger measuring 8.7x5x2.6cms. Cut surface of largest - A muscle is identified measuring 5.6x5.2cms. Random sections from muscle given in - 6795. Cut surface of attached fibrofatty tissue - Multiple (4) lymph nodes identified larger measuring 0.7x0.5cms. Cut surface of larger - Pale white. Section from larger lymph node along with smaller given in - A. Random section from fibrofatty tissue given in - B. Cut surface of other fibrofatty tissue - Multiple (5) lymph nodes identified larger measuring 1.8x0.5cms. Cut surface of larger - Pale white, larger lymph node cut in two given in - C. Section from smaller lymph node - D,E. Random section from fibrofatty tissue given in - F.

Container labelled (1) Buccal mucosa with hemimandible right - Received right sided hemimandibulectomy specimen measuring 8.5x7.6x3.4cms comprising of portion of body, angle and portion of ramus of mandible measuring 4.9cms in length. Also seen 3 tooth in socket lateral aspect (outer) shows attached muscle and fibrofatty tissue inferior aspect shows attached salivary gland and fibrofatty tissue. An ulceroproliferative exophytic growth is seen in the posterior aspect of buccal mucosa near the retromolar trigone measuring 2.2x1.2cms. Surrounding buccal mucosal shows irregular pale white areas. Cut surface - A grey white lesion noted measuring 2.2x1.2x0.8cms, located at a distance of 1.9cm from anterior mucosal margin, 3cm from anterior bony margin (mandible), 1.1cm from posterior mucosal margin, 3.7cm from posterior bony margin (ramus), 0.5cm from superior mucosal margin (involved), 0.3cm from inferior mucosal margin, 1cm from medial alveolar margin, 4cm from deep resected margin (lateral muscle). On lateral aspect muscles appears to be uninvolved. Section from anterior mucosal margin with margin painted - G. Section from posterior mucosal margin with margin painted given in - H. Section from tumor proper with superior margin painted given in - I. Section from tumor proper with inferior mucosal margin painted given in - J. Section from tumor proper given in - K,L. Section from medial alveolar margin with margin painted given in - M. Section from deep resected margin with margin painted given in - N. Section from adjacent mucosa showing irregular pale white areas given in - O. Random sections from muscle attached on the lateral aspect given in - P. Cut surface of the fibrofatty tissue attached on lateral aspect - No lymph node identified. Random section from the fibrofatty tissue given in - Q. Cut surface of - Fibrofatty tissue on the - Inferior aspect shows salivary gland measuring 4x2.6cms. Cut surface of salivary gland - Appears to be uninvolved. Section from salivary gland given in - R. Also noted a nodular area measuring 2.4x1.9cms. Outer surface - Unremarkable. Cut surface - A cyst is noted containing gelatinous material with a pale white solid area noted at the periphery. Also noted areas of necrosis. Entire cut surface cut in two given in - S,T. Multiple (3) lymph nodes identified largest measuring 0.9x0.5cms. Cut surface of larger - Pale brown, larger lymph node along with smaller given in - U. Random section from attached fibrofatty tissue given in - V. Section from anterior, bony margin, posterior bony margin, tumor proper with bony margin - KFD. Anterior bony margin given in - W. Section from tumor with underlying bone given in - X. Section from posterior bony margin given in - Y.

MICROSCOPY:

(1) Container labelled (1) Buccal mucosa with right hemimandible - Multiple sections studied shows an infiltrating tumor arising from severely dysplastic squamous epithelium and arranged in nests and sheets and islands of polygonal cells with pleomorphic vesicular nucleus, prominent nuclei and moderate eosinophilic cytoplasm. Well formed keratin pearls seen. Peritumoral brisk lymphocytic response, karyorrhectic debris and occasional atypical mitotic figures seen lymphovascular invasion seen.

All the margins are uninvolved by tumor. Closest is inferior mucosal margin which is 0.3cm away from tumor. One satellite nodule of tumor is also noted. Three lymph nodes identified and are uninvolved by tumor.

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28-08-2019, 09:52

JUSTICE K.S. HEGDE CHARITABLE HOSPITAL

University Road, Deralakatte - 575018

Tel: 0824 2204471-76 (6 LINES)

Laboratory Test Result

Register No.: 19026933 IP No.: IP009190

Patient Name: SHANTARAM VENKATRAMAN HEGDE

Report No.: L1776272

Age: 65 Years Sex: Male

Dept/Unit: E.N.T/Unit 1

Location: General Ward/C9

(2) Container labelled (2) Right MRND and sternocleidomastoid - Sections studied shows 8 lymph nodes and are uninvolved by tumor. Also noted muscle and is uninvolved by tumor.

CAP SUMMARY - LIP AND ORAL CAVITY:

Procedure: Mandibulectomy Right with Right MRND, Neck (lymph node) dissection

Tumor Site: Buccal mucosa Right

Tumor Laterality: Right

Tumor Focality: Multifocal

Tumor Size: Greatest dimension 2.2×1.2×0.8cms

Tumor Depth of Invasion (DOI) (millimeters): 0.6cm

Histologic Type: Squamous cell carcinoma, conventional

Histologic Grade: G1: Well differentiated

Specimen Margins: Uninvolved by invasive tumor

Lymphovascular Invasion: Present

Perineural Invasion: Not identified

Regional Lymph Nodes: 11 lymph nodes isolated uninvolved by tumor

Pathologic Stage Classification (pTNM, AJCC 8th Edition)

Primary Tumor (pT): pT2: Tumor ≤2 cm, DOI >5 mm and ≤10 mm

Regional Lymph Nodes (pN): pN0: No regional lymph node metastasis

Additional Pathologic Findings: Keratinizing dysplasia, severe (carcinoma in situ)

IMPRESSION & SUMMARY:

(1) Histological features are suggestive of Well differentiated Squamous Cell Carcinoma - Right buccal mucosa - TNM - pT2 No.

(2) Margins - Uninvolved by tumor. Closest is inferior mucosal margin 0.3cm away from tumor.

(3) Lymph nodes - 11 lymph nodes uninvolved by tumor

(4) Lymphovascular invasion - Identified.

(5) Perineural invasion - Not identified.

(6) Additional findings - Keratinising severe dysplasia with satellite nodule within the specimen

(7) TNM - pT2 No.

Entered By: KRIPA, chandini

Dr. Jayaprakash Shetty K, M.D(Path)
Professor & HOD, Pathology

Dr. Krishna Prasad H. V, DCP, M.D (Path)
Assistant Professor

Printed On: 28-08-2019, 09:47 AM

Report Generated On: 08/8/19, 02:36 PM

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**Justice K.S.Hegde Charitable Hospital**

[A Unit of K.S.Hegde Medical Academy]
Deralakatte, Mangalore - 575018, Karnataka, India. Tel: 0824-2204471-76, Fax: 0824 - 2204016
Website: www.nitte.edu.in E-mail: kshema_hospital@nitte.edu.in

Discharge Summary

Patient Name	: SHANTARAM VENKATRAMAN HEGDE	Patient Status	: Recovering
Register No.	: 19026933	Referred By	: -
Age	: 65 Yrs, Sex: Male	Room No.	: C9
Address	: Bommanalli Sirsi, Post Bommanalli, Karwar	Discharge Date & Time	: 12-08-2019
Admission Date & Time	: 12-07-2019, 09:43 AM	Doctor Name	: -
Department/Unit	: E.N.T/Unit I		
Referred To	: -		

DIAGNOSIS:**MALIGNANCY RIGHT BUCCAL MUCOSA****COMPLAINTS:**

C/o burning sensation in the mouth since 2 months

PAST HISTORY:

No h/o any surgery

Not a k/c/o DM, HTN, BA, TB

FAMILY HISTORY:

Nothing significant

GENERAL EXAMINATION:

Moderately built and nourished

BP: 140/80 mm of Hg

Pulse : 82 bpm

No pallor, icterus, cyanosis, clubbing, lymphadenopathy, edema

EAR:

NAD

NOSE:

DNS to the left side

Right HIT

THROAT:

NAD

ORAL CAVITY:

Lips- Normal

Teeth-Nicotine stained

Mouth Opening - Normal

Buccal mucosa- 2x 1.5cm ulcerative proliferative lesion noted involving right buccal mucosa, situated 0.5cm from right retro molar trigone and 1cm from the anterior pillar and 3cm from the angle of the mouth extending to upper gingivo buccal sulcus

Floor of mouth- Normal

Palate-Normal

ILS:

B/L vocal cords mobile and edematous

NECK:

A single 3x2 cm non tender lymph node in the right sub mandibular region, mobile horizontally

Skin normal

INVESTIGATIONS:

02-08-2019 USG Abdomen and Pelvis

Calcified lesions noted in the right lobe of liver ? Calcified granuloma

Grade 1 Prostatomegaly with medial lobe hypertrophy and post void bladder residue of 45cc

01-08-2019, 03:05 PM Urine Analysis Complete Specific Gravity - 1.010, PH - 6.0, Protein - NIL, Glucose - NIL, Ketone

Bodies - NIL, Urobilinogen - Normal, Bile pigment - NIL, Bile Salt - NIL,

Blood - NIL, Epithelial Cells - 2-4/HPF, Casts - NIL, Crystals - NIL, Others, Erythrocytes - NIL, Leucocytes - 3-5/HPF

12-07-2019, 02:06 PM Platelet Count - 227000, Neutrophils - 79, Lymphocytes - 18,

Eosinophils - 03, Haemoglobin - 16.3, Total Leucocyte Count (TLC) - 6900, Erythrocyte Sedimentation Rate (ESR) - 05

12-07-2019, 12:26 PM Glycated Haemoglobin - 5.17, Estimated Average Glucose - 102.54, Glucose Plasma - Random -

101, Electrolytes, Sodium - 138, Potassium - 5.13, Chloride - 97.6, Creatinine - 1.14, Blood Urea - 31.2

22-07-2019, 09:45 PM Specific Gravity - 1.025, PH - 6.0, Protein - NIL, Glucose - NIL, Ketone Bodies - NIL, Urobilinogen -

Normal, Bile pigment - NIL, Bile Salt - NIL, Blood - NIL, Epithelial Cells - 1 - 2/HPF, Casts - NIL, Crystals -

NIL, Others, Erythrocytes - NIL, Leucocytes - 2-4/hpf

26-07-2019, 11:15 PM Blood Group With RH Type - " O " POSITIVE

26-07-2019, 11:48 PM Prothrombin Time (PT), Test - 14.3, Control - 14.1, INR - 1.02

27-07-2019, 08:19 PM Electrolytes, Sodium - 138, Potassium - 4.32, Chloride - 102.3, Creatinine - 1.17, Blood Urea - 22.0

27-07-2019, 08:43 PM Neutrophils - 94, Lymphocytes - 05, Eosinophils - 01, Total Leucocyte Count (TLC) - 12900, Packed

Cell Volume (PCV) / Hct - 41.2, Haemoglobin - 13.7

28-07-2019, 12:11 PM Total Leucocyte Count (TLC) - 13300, Packed Cell Volume (PCV) / Hct - 45.7, Haemoglobin - 15.2

22/6/19 HISTOPATHOLOGY - Histological features are suggestive of Carcinoma in-situ with early invasion - Right buccal mucosa

5/7/19- FNAC-FEATURES ARE SUGGESTIVE OF METASTATIC SQUAMOUS CELL CARCINOMA - RIGHT CERVICAL LYMPHNODE.

8/8/2019-HISTOPATHOLOGY-

(1) Histological features are suggestive of Well differentiated Squamous Cell Carcinoma - Right buccal mucosa - TNM - pT2 No.

(2) Margins - Uninvolved by tumor. Closest is inferior mucosal margin 0.3cm away from tumor.

(3) Lymph nodes - 11 lymph nodes uninvolved by tumor

(4) Lymphovascular invasion - Identified.

(5) Perineural invasion - Not identified.

(6) Additional findings - Keratinising severe dysplasia with satellite nodule within the specimen

13-07-2019, 12:58 PM CECT NECK

- Enhancing thickening of right upper posterior buccal mucosa with retromolar trigone extension, suspicious erosion of maxillary alveolar process and enlarged right Ib lymph node. Suggested HPE correlation.

TREATMENT GIVEN:

RIGHT COMPOSITE RESECTION + RIGHT HEMI MANDIBLECTOMY + RIGHT TYPE 2 MRND + PMMC

RECONSTRUCTION ON 27-07-2019 UNDER GA

INTRA OP FINDINGS:

Ulcerative lesion over the right buccal mucosa extending to the right retromolar trigone

HOSPITAL STAY:

65 year old male patient came with the above mentioned complaints and was clinically radiologically and histopathologically diagnosed to have WELL DIFFERENTIATED SQUAMOUS CELL CARCINOMA RIGHT BUCCAL MUCOSA - pT2 No.

Patient had itching in the fingers for that Dermatology opinion was asked and he was diagnosed to have allergic contact dermatitis and their advice followed urology opinion was asked in view of incomplete voiding of urine and followed their advice.

He underwent RIGHT COMPOSITE RESECTION + RIGHT HEMI MANDIBLECTOMY + RIGHT TYPE 2 MRND + PMMC RECONSTRUCTION ON 27-07-2019 UNDER GA. Histopathology showed well differentiated squamous cell carcinoma and Lymph nodes and tumor margins negative for tumor. Now the patient is stable and being discharged with the following advice.

ADVICE ON DISCHARGE:

D-PROTEIN POWDER 2 tsp in milk 1-0-1 x (to continue)

Syp. LYCOCELL 2tsp 0-1-0 (to continue)

T. RANTAC 150mg B/F 1-0-1 x 10 days

T. ULTRACET 1 sos x 10 tabs

T SILODAL D 0 - 0 - 1 X 3 weeks
CANDID MOUTH PAINT for LA x 1 week
BETADINE GARGLE 1-0-1 X 5 DAYS

Review in ENT 1 OPD after 2 weeks on 26/8/19(Monday)
ENT 1 OP days are Mon/Wed/Fri
Review in urology opd after 3 weeks

∴
We the patient / patient attendants have been explained regarding the advise on discharge and follow up instructions in our own words and language by Dr Sreesan Drs signature : [Signature]

Patient / patient attendants name & signature : [Signature]

Dr Sreesan P.S
KRL20150000831KTK

HELP LINE:
9353904837

Entered By: Dr.Karan

Checked By: Dr.sachin

DR.SATHEESH KUMAR BHANDARY
Vice Chancellor

Dr. Rajeshwary
Prof. & HOD

DR.VADISH BHAT
Professor

DR. MARINA SALDANHA
ASST. PROF.

DR. GAUTHAM M.K
Assistant Professor

DR. SHASHANK KOTIAN
Senior Resident

Dr. GAUTHAM M.K
Reg. No. 84601 (KMC)